



WNHS PROCEDURE	
Clinical Handover – Mother Baby Unit (MBU)	
Scope (Staff):	Mental health staff working in WNHS
Scope (Area):	Mother Baby Unit (MBU)

1. Aim

The purpose of this guideline is to ensure relevant, accurate and structured clinical handover occurs at the required times along the patient journey and supports safe patient care, consistent with the [WA DoH Clinical Handover Policy](#). This includes on referral, initial assessment, treatment, throughout inpatient stays and when transfer of care takes place and discharge to other care providers.

2. Risk

Non-compliance with this policy will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☐
Breach National/State/Mandatory Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Impact financial operations	☐
Damage NMHS reputation	☐	Impact on Staff Safety	☐
Other	☐		

3. Key Points

- Effective communication between health workers has been recognised as an essential element in best practice patient care and in the reduction of adverse events.
- Clinical handover occurs using a process by which information and responsibility is transferred, risk is identified, and continuity of care is facilitated.
- There is a requirement for clinical handover to follow a structured process and for this purpose the iSoBAR mnemonic is utilised (Identity, Situation,

Observations, Background, Agreed plan, Read back).

iSoBAR: a handover “how to”

i	IDENTIFY	Introduce yourself and your patient
S	SITUATION	Why are you calling? Briefly state the problem
O	OBSERVATIONS	Recent vital signs and clinical assessment
B	BACKGROUND	Pertinent information related to the patient
A	AGREED PLAN	What needs to happen? Assessment of the situation
R	READ BACK	Clarify and check for shared understanding. Who is responsible for what and by when?



Identify



Situation



Observations



Background



Agreed plan



Read back

- Consumers and their carers are involved in the clinical handover process where possible/appropriate.
- Clinical handover occurs between clinicians who are transferring care responsibilities at multiple points of time during admission or care pathway including:
 - a. Nursing shift handover
 - b. Referring agency to inpatient team
 - c. Advice of admission and discharge plan to GP
 - d. Relevant information to other key providers of care e.g. Child health nurse (will require consent to share information)
 - e. Handover to after hours and weekend medical cover for patients at risk/of clinical concern

4. Process

On admission

- The MBU Admin Assistant will inform the patients GPs of the admission to MBU by a standardised letter. This will be posted to the nominated GP.
- The CNC will inform the Child Health Nurses of the admission to MBU by emailing the [Special Referral Form](#) to Child Adolescent Health Service

(CACH) Birth Notifications global email box -

BirthNotificationsCDIS.CACH@health.wa.gov.au

- For WACHS patients all referrals sent to WACHS area office email:
AreaOfficePopulationHealth.WACHS@health.wa.gov.au
- [Child health MBU handover flowchart](#)

Nursing Handover

- Nursing handover occurs on three occasions over a 24 hour period.
- All receiving shift nurses are expected to attend handover.
- The allocated nurse records the presentation of the patient during their time allocated to the patient in the medical record, signing and dating each entry.
- The allocated nurse will stamp the notes with the MDT Clinical Handover stamp at the end of the entry for the span of duty.
- The allocated nurse will ensure that iSoft is updated prior to the end of their shift.
- Any issues of concern must be escalated to the shift coordinator
- The allocated nurse will handover their patients, using the iSoBAR mnemonic including the following information:
 - Patient's Name and Baby's Name
 - Room number
 - Reason for admission (briefly outlining difficulties/issues and any significant history)
 - Current risks, current observation level for mother and baby, leave, and leave arrangements
 - Physical observation
 - Medication chart - both nurses to check the chart/s to ensure all medications have been administered as prescribed
 - Management plan - considering the key focus of care, any updates or changes or issues. An outline of the patient's presentation during the shift
 - The patient's current whereabouts and estimated time of return if on leave
 - Items for follow up by the receiving nurse.
- Both the receiving nurse and the allocated nurse handing over will sign the stamped area indicating handover has been accepted and understood
- The shift co-ordinator will allocate patients to the incoming nursing staff according to the handover information, ensuring the most appropriate skill set is utilised and any significant risks are highlighted.
- This time is also to be used for any further patient information needing to be shared with all staff.

- This process is repeated on all shifts thereafter.
- Day to afternoon shift handovers will include a “huddle” when the day shift coordinator will conduct a brief stand –up meeting by the patient board that highlights any patients of concern, risk issues and significant management changes.
- Day to afternoon shift handover will include a face to face handover with the patient, carers, and family members, where possible.
- This handover is to be documented in the medical record and identified by the “patient engagement” sticker
- Day to afternoon shift will be a day shift allocated nurse to afternoon allocated nurse handover of care.

Daily MDT Handover

- Daily handover occurs at 9am Monday to Friday in the nursing office at the patient board.
- The handover will include of a “huddle” where the day shift coordinator will conduct a brief stand –up meeting by the patient board that highlights any patients of concern, risk issues and significant management changes as well as providing a short overview of the past 24 hours of all patients (any detailed discussions of the patient can occur outside of this meeting).
- All available members of the MDT are expected to attend.
- Any patients of concern, risks, new issues to be handed over to the medical team ASAP – if medical staff not able to attend the 9am huddle.
- Urgent concerns that require rapid intervention should be escalated to the duty psychiatrist.

Referral Handover

- All members of the MDT are expected to attend.
- The CNM or CNC presents the information obtained for each new referral in the iSoBAR format.
- The CNM or CNC provides an update of any referrals that are pending.
- Following the handover the CNM or CNC will action any decisions.

Weekly Ward Round

- Weekly ward round occurs at 9am every Thursday.
- All members of the MDT are expected to attend.
- Prior to the ward round the allocated nurses will have reviewed the patient goals of admission psolis plan in consultation with the patient.

- A risk assessment will be completed by nursing staff on any patient who has not had a risk assessment undertaken over the previous week.
- New patients are presented in detail giving a psychiatric, medical, and social history, as well as the presenting features of current illnesses.
- An initial diagnosis will be given and an expected length of stay will be determined.
- All other patients will be discussed outlining their progress over the previous week from a medical, nursing, social work and psychological perspective.
- Patients will have been reviewed by members of the MDT during the week, engaging with them and their families to develop goals and plans of care.
- A plan will be formulated for the coming week outlining leave, observation levels and treatment considering identified risks and will be recorded in the medical record.
- Discussion and key points are documented in the medical record along with participants at the review.
- Nursing staff will provide feedback from the patient to the team as per discussions with the patient from the review of their identified goals as well as a brief overview from a nursing perspective escalating if there are any concerns

Medical handover to on call/after-hours clinicians

- Weekend staff to be provided with inpatient list on Friday afternoon.
- Verbal communication to registrar and/or consultant on call for relevant patients of concern, and in particular those requiring face to face reviews. This applies to weekdays after hours, weekends, and public holidays. All patients under the Mental Health Act are to be reviewed daily. All new inpatients are to be reviewed within 24 hours of admission.
- In addition to documenting in the notes, after hours staff to provide email OR verbal handover of their assessment and intervention.

Discharge

- Patients/carers/PSP/families will be involved in the discharge planning and handover of care
- Prior to leaving MBU all patients will be provided with a copy of their discharge summary and PSOLIS discharge plan. If either plan has not been completed, they will be posted to the patient as soon as possible.
- MBU will provide a discharge summary to the nominated GP and Community Mental Health Teams on discharge. These will be faxed on discharge. MBU will provide a Care Transfer Summary and updated risk assessment to Community Mental Health Teams. The CNC will contact services prior to discharge and fax through paperwork.

- A verbal handover will be provided to community teams if required for complex patients.
- The CNC will provide the child health nurse an isobar summary of any care provided focusing mostly on the care of the baby. Refer to [Child health MBU handover flowchart](#)

Related internal policies, procedures and guidelines

[WA DoH Clinical Handover Policy](#)

[Women's and Perinatal Mental Health Referral and Management Guideline](#)

Useful resources

[Child health MBU handover flowchart](#)

[Special Referral Form](#)

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NSQHS Standards Applicable:	☐  Std 1: Clinical Governance ☐  Std 5: Comprehensive Care		☐  Std 6: Communicating for Safety ☐  Std 8: Recognising and Responding to Acute Deterioration		

National Standards for Mental Health Services	☐ Std 1: Rights and Responsibilities ☐ Std 2: Safety ☐ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management	☐ Std 9: Integration ☐ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry
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The health impact upon Aboriginal people has been considered, and where relevant incorporated and appropriately addressed in the development of this policy (insert ISD Number). (Please refer to the Aboriginal Health Impact Statement and Declaration for Department of Health and Health Service Provider Guidelines – please delete once you have completed this).

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